About this guide

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Clinical manuscript review

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Reviewer contributions and credentials are supplied by the author. Dr. Salwa Ismael Hegazy reviewed the revised manuscript and amendments; Ahmed Amin's contribution concerns the original manuscript. No institutional endorsement or empirical validation is claimed.

Purpose and scope

Every facilitator knows the moment: the skill is on the board, and the room has gone somewhere else. This guide is for that moment. It is written for qualified facilitators and supervised trainees who run adult skills groups in IOP and PHP programs. Each fictional case gives you a first response, choices for what happens next, a way back to the planned skill, and sample documentation.

The cases are set in rolling-admission groups with planned 50-minute sessions. Most take place in the room; four take place by video. Group size, staffing, and level of care change from case to case, as they do in real programs, so adjust each one to your own setting.

The worked skills come from dialectical behavior therapy (DBT). The practice examples are short and are meant to support a lesson you are already teaching. They do not replace a DBT curriculum, your training and supervision, or your program's procedures. The documentation examples are starting points, not complete notes, and they are not written to support medical necessity, payment, or legal compliance.

All names, cases, and dialogue are invented. The references support the principles behind each response; the scripts themselves have not been tested as a clinical protocol. The cited authors and organizations have not sponsored or endorsed this book. Each scenario stands on its own.

Acute safety and virtual groups

Scenarios 33 to 35 cover a stated plan to end one's life, apparent intoxication, and a threat toward another member. Scenarios 36 to 39 cover groups held by video. They show a facilitator's first response: what to say, how to stay with the group, and how to hand off. They are not a suicide risk assessment, crisis protocol, or security procedure, and they do not replace your program's emergency procedures.

Permission to print the cards

Program-use permission. The purchasing professional or program may make internal copies of the designated practice pages, session plans, cards and blank implementation forms for its own staff and clients directly served. Staff may use these copies within that program; a separate purchase for each staff member is not required. This permission does not allow resale, public uploading, external distribution of blank files, or distribution of the complete digital books. Other uses require written permission from Thoughtkind Press. Keep completed clinical information in approved systems. The cards and staff implementation forms are professional references, not client handouts. Permission covers original material in this publication only, not cited publishers' materials.

Program preparation and local response

Staff-only implementation forms. Confirm permitted roles, actual competence, coverage and local approval before use. The scenarios are examples, not comprehensive emergency protocols.

Program setup: decisions before the first group

This pack turns the implementation questions into writable decisions. It is not a completed local SOP. Clinical leadership approves the local routes, permitted roles, staffing and record procedures before use. Keep it beside the session plan, not in a client workbook.

Program / site / jurisdiction / version / review date

_____
_____
_____

Clinical approver and authorized roles; relevant license or supervision requirements

_____
_____
_____

Group purpose, maximum size and minimum staffing; reason for these choices

_____
_____
_____

Named backup, contact method, hours and alternative if unavailable

_____
_____
_____

Before opening a group, verify coverage for both the person needing support and the remaining group. If the planned backup is absent, obtain an approved alternative or change/postpone the session under site policy. Do not improvise a routine group without the coverage it requires. An emergency that occurs anyway calls for the emergency route immediately.

Local safety addendum 1: urgent routes

Complete each route with a named role, working contact, immediate action, coverage of the group, and handoff destination. Test contact details privately. Do not put participant identities on this blank staff reference.

Medical emergency / suspected ingestion / breathing or consciousness concern

_____
_____
_____

Suicide or self-harm concern: assessor / emergency route / staff coverage

_____
_____
_____

Threats or violence: protection / security or emergency route / protected record

_____
_____
_____

Intoxication, withdrawal or possible medication reaction: medical route

_____
_____
_____

Acute psychosis, mania, severe disorganization or eating-related medical concern: qualified assessment route

_____
_____
_____

A routine worksheet is not the response to an acute medical or psychiatric concern. Activate the appropriate local response promptly; do not wait an arbitrary number of minutes for a callback. The program determines monitoring, transfer and return arrangements.

Local safety addendum 2: continuity and privacy

Remote: verified current location, safe callback and local responder; disconnection plan

_____
_____
_____

Safeguarding / mandated reporting / duty-to-protect consultation route

_____
_____
_____

Private space and staff coverage; leaving / rejoining arrangements

_____
_____
_____

Records owner, approved system, access, retention and secure disposal policy

_____
_____
_____

Incident notification, review owner and follow-up deadline

_____
_____
_____

The sending staff member states the concern, actions taken, uncertainties, next action and contingency. The receiving staff member confirms understanding and accepts the specified responsibility; record who accepted what and when. Sending a message alone does not complete a handoff. See AHRQ, Tool: Handoff.

Staff explain applicable confidentiality limits and their reporting duties before participation. Ask members to protect privacy, while explaining that staff cannot guarantee other members will do so. Resolve profession- and jurisdiction-specific questions through the program.

Fit for this group: a private clinical decision

Assess the person and the actual group task together. A diagnostic label, neurodivergence, a communication difference or limited literacy is not an automatic exclusion. AGPA discusses the interaction of client fit, group composition and leader competence.

Person / approved identifier / date / responsible clinician

_____
_____

Relevant care goal, preferred participation and access or interpretation needs

_____
_____
_____

Current capacity for this task; acute concerns needing individual assessment first

_____
_____
_____

Decision: proceed / adapt / defer to another service; reason and alternative support

_____
_____
_____

Staffing and space needed; person responsible; review / return criteria

_____
_____
_____

Acute withdrawal, possible medical instability, severe mood elevation, psychosis-related distress or severe disorganization require assessment of current needs. Do not equate a calm appearance, a rating, attendance or worksheet completion with suitability or permission to return.

One-page code implementation plan

Copy this form for each code in use. Consult the code matrix for its specific competence focus. Approval belongs to the adopting program; this blank form does not certify anyone.

Code / title / purpose linked to care plans / approved version

_____
_____

Permitted lead roles and demonstrated task competencies

_____
_____

Co-facilitator or direct supervisor; when they must be present

_____
_____

Local SOP / stop signs / immediate action / qualified recipient and contact

_____
_____
_____

Group coverage / private support / accepted handoff and return criteria

_____
_____
_____

One demonstration / complete practice / debrief / close; access alternatives

_____
_____
_____

Clinical approver / signature or approved-system confirmation / date / review date

_____
_____

Minimum viable session: keep the useful work

Choose one aim and one complete practice. A short group should still let a person try something, notice what happened and leave with a support route. Do not try to deliver a whole long plan faster.

Shorter skills, psychoeducation and PHP routes (minutes: opening / demonstration / one complete practice / debrief / close): 30 = 4 / 4 / 12 / 5 / 5; 45 = 6 / 9 / 18 / 7 / 5; 50 = 7 / 10 / 20 / 8 / 5; 60 = 8 / 12 / 25 / 10 / 5; 90 = 12 / 18 / 35 / 15 / 10. Choose one focused task; shorten explanation and extra examples before cutting the practice or close. Safety checks and access adaptations apply throughout. These are facilitation examples, not billing rules. Use the separate process-group route for PR work.

Today's code / care-related aim / time available

_____
_____

Fictional demonstration in ordinary words; materials already available

_____
_____
_____

One complete practice and an equivalent spoken, shown or observed option

_____
_____
_____

Debrief: what fit, what did not, what needs changing?

_____
_____

Close: optional next step, question or support; owner of unresolved concerns

_____
_____
_____

Example: for a clear request, show one short request, let the member change the words, try it once, then ask whether it fits their actual relationship and circumstances. A second example is optional; the first complete attempt matters more.

Staff handoff: confirm who owns the next action

Professional use only. Complete through the approved private system. A Passport or client sheet is not the clinical handoff.

Sender / receiver / time / approved identifier / code

_____
_____

Observed behavior and attributed client report; current concern and uncertainty

_____
_____
_____

Intervention and adaptation actually used; response actually observed or reported

_____
_____
_____

Relevant care-plan or safety-plan location; pending contact or referral

_____
_____
_____

Next action / named owner / timeframe / contingency if no response

_____
_____
_____

Receiver's confirmation of understanding and accepted responsibility / time

_____
_____
_____

Avoid copied claims of engagement, recovery or consent that were not established. Exclude unrelated peer histories and unnecessary identifiers. Include necessary, relevant safety information in the appropriate protected record as permitted or required; do not apply a blanket ban that erases essential facts.

QI record 1: delivery and access

This is a local improvement form, not a validated outcome scale. Keep program review data de-identified where possible and separate from the clinical record.

Review period / owner / groups sampled / sampling method

_____
_____

Attendance: attended opportunities / offered opportunities; define exclusions

_____
_____

Retention: your local definition, eligible denominator and count; reasons known or unknown

_____
_____

Delivery: Bridge, meaningful choice, one complete practice, appropriate safeguards, supported close; completed / adapted / not delivered / not applicable

_____
_____
_____

Access barrier, declined activity or adaptation; what the team will change

_____
_____
_____

Supervision discussion / action owner / date to check the change

_____
_____
_____

Report the denominator and missing information. Attendance is not improvement; a fidelity tick is not evidence of clinical benefit. Review the work with supervision rather than scoring staff from an isolated incident.

QI record 2: experience, concerns and goals

Period / reviewer / data source / privacy arrangements

_____
_____

Adverse event or near miss: category, immediate action, responsible reviewer; detailed protected incident record location

_____
_____
_____

Optional participant experience: what helped? what felt unhelpful or unsafe? preferred change or private route

_____
_____
_____

Individualized goal: measure chosen in the care plan, baseline, follow-up, missing data and interpretation by the responsible clinician

_____
_____
_____

Team learning: decision, owner, completion date, next review

_____
_____
_____

Offer anonymous or private feedback where feasible and explain its limits. Passing or missing feedback does not mean satisfaction. Do not infer safety, effectiveness or validated outcomes from this untested improvement pack. Do not pool identifiable information into shared training documents.

Localization and rehearsal record

Jurisdiction / local language / clinical and privacy approvers

_____
_____

Verified emergency and crisis contacts; service hours; safe remote-response route

_____
_____
_____

Applicable scope, reporting, consent, confidentiality and record requirements; policies checked and review date

_____
_____
_____

Translation / interpreter / cultural and sensory adaptations checked with intended users

_____
_____
_____

Rehearsal: lead, backup and group coverage; contact test; interrupted handoff; no-backup contingency

_____
_____
_____

Issues found, changes made, supervisor authorization and next review

_____
_____
_____

Practice with fictional information. Rehearsal is preparation, not proof of competence or clinical effectiveness. A small supervised local introduction can identify usability problems; record what actually happened rather than claiming a pilot has already succeeded.

Role and competence matrix: all 39 scenarios

For every scenario the program documents the permitted lead role, competence evidence, supervision or co-leading, and backup coverage on the code implementation form. Reading a case does not qualify a person to deliver assessment, prescribing, safeguarding or emergency care. Use qualified staff and the site's approved role assignments.

01 / One member needs more time than the group can offer

Group facilitation, consent, boundaries and accessible instruction; competence in the planned STOP lesson.

02 / Nobody answers the opening question

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Describe lesson.

03 / A disagreement becomes personal

Group facilitation, consent, boundaries and accessible instruction; competence in the planned GIVE lesson.

04 / A member declines the activity

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Check the Facts lesson.

05 / A new member joins midway through the cycle

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Problem Solving lesson.

06 / The facilitator makes a harmful assumption

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Validation lesson.

07 / A member repeatedly interrupts other speakers

Group facilitation, consent, boundaries and accessible instruction; competence in the planned One Mindfully lesson.

08 / Advice takes over another member's turn

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Validation lesson.

09 / A member cannot use the written worksheet

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Describe lesson.

10 / A tired member struggles to stay with the lesson

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Observe lesson.

11 / The room goes quiet after a painful disclosure

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Validation lesson.

12 / The available language support does not meet the task

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Describe lesson.

13 / Check-in leaves too little time for practice

Group facilitation, consent, boundaries and accessible instruction; competence in the planned One Mindfully lesson.

14 / A comment targets a member's identity

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Validation lesson.

15 / Laughter interrupts a member's contribution

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Check the Facts lesson.

16 / Several members question whether a boundary is fair

Group facilitation, consent, boundaries and accessible instruction; competence in the planned DEAR MAN lesson.

17 / Members ask for graphic details of a painful event

Group facilitation, consent, boundaries and accessible instruction; competence in the planned STOP lesson.

18 / A member asks for diagnosis or medication advice

Medication-question boundaries, prompt private clinical routing and accepted handoff; prescribing and diagnostic decisions remain with the qualified clinician.

19 / A member asks for a guarantee that the skill will work

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Pros and Cons lesson.

20 / Returning members say they have done this skill already

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Effectively lesson.

21 / Arrivals and departures interrupt the session

Group facilitation, consent, boundaries and accessible instruction; competence in the planned STOP lesson.

22 / A familiar member is unexpectedly absent

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Validation lesson.

23 / A practice exercise increases distress

Consent, sensory or environmental adaptation, stopping unwanted practice and recognizing when qualified clinical or medical review is needed.

24 / The room becomes difficult to tolerate

Consent, sensory or environmental adaptation, stopping unwanted practice and recognizing when qualified clinical or medical review is needed.

25 / A suggested skill does not fit a real constraint

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Problem Solving lesson.

26 / A member raises a legitimate concern about the intervention

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Check the Facts lesson.

27 / A member points out an inconsistent boundary

Group facilitation, consent, boundaries and accessible instruction; competence in the planned DEAR MAN lesson.

28 / Co-facilitators give conflicting directions

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Effectively lesson.

29 / A member leaves the room during tension

Departure, re-entry or ending arrangements; verify current needs, group coverage and named staff responsibility.

30 / A member returns after individual support

Departure, re-entry or ending arrangements; verify current needs, group coverage and named staff responsibility.

31 / The same disruption recurs across sessions

Group facilitation, consent, boundaries and accessible instruction; competence in the planned Problem Solving lesson.

32 / The session must end while tension remains

Departure, re-entry or ending arrangements; verify current needs, group coverage and named staff responsibility.

33 / A member says they plan to end their life

Training and rehearsed role in the relevant local urgent-response procedure, with available qualified clinical or emergency support and group coverage. Program specifies supervision/co-leading before use; trainees do not independently assume assessment or emergency authority.

34 / A member appears intoxicated

Training and rehearsed role in the relevant local urgent-response procedure, with available qualified clinical or emergency support and group coverage. Program specifies supervision/co-leading before use; trainees do not independently assume assessment or emergency authority.

35 / A member threatens another member

Training and rehearsed role in the relevant local urgent-response procedure, with available qualified clinical or emergency support and group coverage. Program specifies supervision/co-leading before use; trainees do not independently assume assessment or emergency authority.

36 / A virtual member hints at danger, then disconnects

Training and rehearsed role in the relevant local urgent-response procedure, with available qualified clinical or emergency support and group coverage. Program specifies supervision/co-leading before use; trainees do not independently assume assessment or emergency authority. Verify remote location, safe callback, privacy and disconnection arrangements.

37 / Someone else is in a virtual member’s room

Training and rehearsed role in the relevant local urgent-response procedure, with available qualified clinical or emergency support and group coverage. Program specifies supervision/co-leading before use; trainees do not independently assume assessment or emergency authority. Verify remote location, safe callback, privacy and disconnection arrangements.

38 / A member joins while driving

Training and rehearsed role in the relevant local urgent-response procedure, with available qualified clinical or emergency support and group coverage. Program specifies supervision/co-leading before use; trainees do not independently assume assessment or emergency authority. Verify remote location, safe callback, privacy and disconnection arrangements.

39 / A private chat message asks you to keep a secret

Training and rehearsed role in the relevant local urgent-response procedure, with available qualified clinical or emergency support and group coverage. Program specifies supervision/co-leading before use; trainees do not independently assume assessment or emergency authority. Verify remote location, safe callback, privacy and disconnection arrangements.

Situations outside the 39 worked cases

These are routing reminders, not additional treatment algorithms. Stop routine teaching when current needs exceed it; use the locally approved response and qualified staff. Do not diagnose from a brief group observation.

Suspected overdose, seizure, severe withdrawal or medication reaction

Activate the appropriate urgent medical/emergency route for the current presentation. Use trained staff, local procedures and group coverage; do not ask peers to treat or monitor.

Severe confusion, cognitive impairment or altered consciousness

New or significant change requires qualified assessment. For ongoing access needs, arrange a suitable supported format and individual fit review.

Child protection, abuse, duty to protect or other reporting question

Use the program's designated safeguarding/clinical consultation route promptly and follow applicable requirements. Do not conduct a public investigation or promise secrecy.

Recording, screenshots or exposed remote communication

Stop the privacy exposure using the platform and program procedure; arrange a safe private route. Do not demand sensitive disclosure on a possibly monitored device.

Under-18 participant or a dual relationship

This adult series does not supply a minor's protocol. Confirm consent, safeguarding, boundaries and an appropriate service before routine participation.

Harassment, threat, unsafe room or evacuation

Protect the affected person, maintain safe exits and activate the local security/emergency or evacuation route when indicated. Do not improvise restraint.

Facilitator overwhelm or unavailable backup

Request approved coverage and supervision. Follow the planned pause/transition route; a lone facilitator should not pretend that two simultaneous care responsibilities are covered.